

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

2

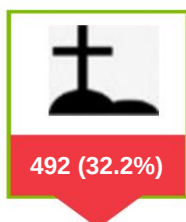
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°050/MVB_02/07/2026

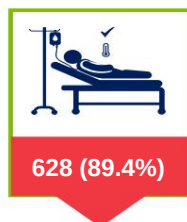
Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (36)	- Ituri (24/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima and Lolwa. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	July 3, 2026
Publication date	July 4, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



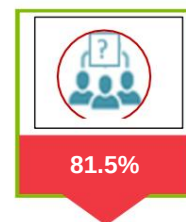
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 2 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

Twenty -six (26) new confirmed cases, including 9 community deaths, were reported on July 3, 2026, in the provinces of Ituri (21 cases, including 6 deaths) and North Kivu (5 cases, including 3 deaths). These confirmed cases originate from the health zones of **Rwampara (11), Bunia (5), Mongbwalu (3), and Mangala (2)** in Ituri, and from the health zones of Butembo (2), **Musienene (1), Masereka (1), and Beni (1)** in North Kivu.

- **Ten (10)** deaths were recorded, including 8 in Ituri among patients with Ebola virus disease in Ebola treatment centers; (3 in Mongbwalu, 2 at ISTM Nyakunde, 1 in Bunia, 1 in Elikya and 1 in SOTA. For North Kivu (2 in Katwa).
- **Ten (10)** patients with Ebola virus disease, including 9 in Ituri province (3 at CTE ISTM, 3 at CTE Mongbwalu, 2 at CT Bambu, 1 at CTE Nyakunde) and 1 at CTE Katwa in North Kivu province, have been declared cured after obtaining negative control tests.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of

nearly 15 million inhabitants, with massive internal displacements and cross-border flows to neighboring countries.



Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of July 3, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of July 3, 2026.

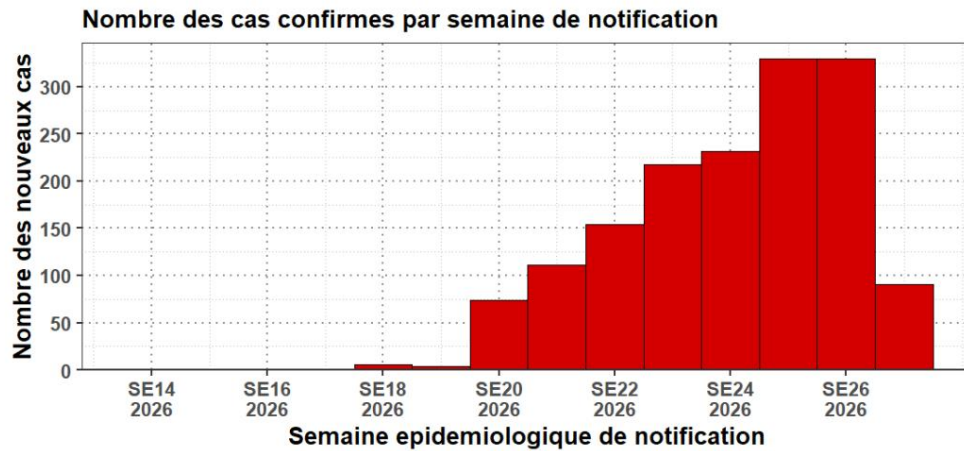
Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24 hours)
Ituri	1,387	411*	29.6%	24 out of 36 (66.7%)	21
North Kivu	138	80*	58.0%	11 out of 34 (32.3%)	5
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	1,528	492	32.2%	36 out of 104 (34.6%)	26

*8 deaths in Ituri and 2 deaths in North Kivu were recorded among patients with Ebola virus disease (previous confirmed cases) in the CTE/CT

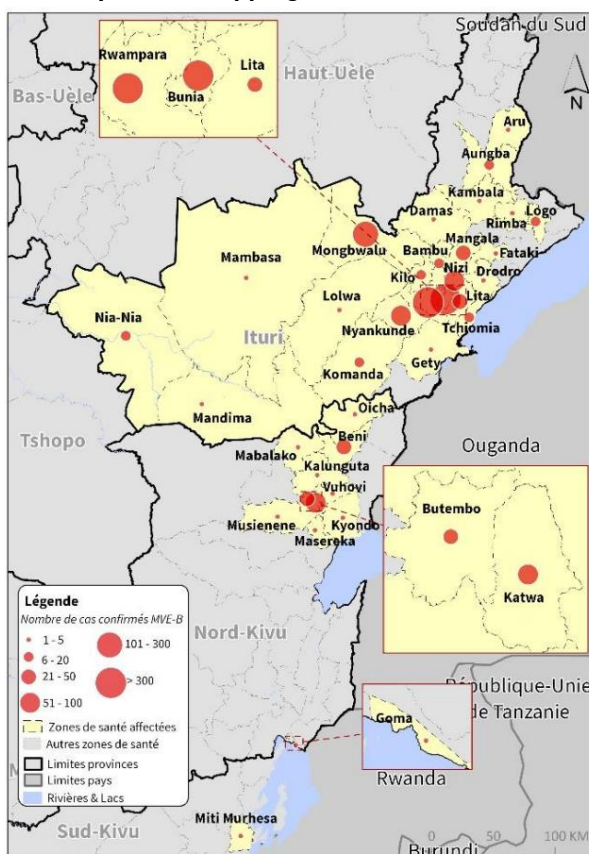
At the end of the day on July 2, 2026, no new health zones were affected. The number of affected health zones remains at 36/104 (section 3.1). Since May 26, 2026, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission.

3.3 Time-Based Analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease, although the number of cases appeared to be stable during epidemiological weeks 24 and 25. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capacities) contributed to early detection, confirming continued and increased community transmission in week 25. Compared to all other weeks in which cases have been recorded since the beginning of the epidemic, weeks 25 and 26 represent

up to now the peak, i.e. more than 300 cases per week.



3.4 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of July 3, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (July 3, 2026)

Province / Health Zone	Cumulative confirmed cases	Cumulative confirmed deaths (n)	Case fatality rate (CFR%)
ITURI	1387	411	29.6
Bunia	431	112	26.0
Rwampara	333	69	20.7
Mongbwalu	276	139	50.4
Nyankunde	95	17	17.9
Nizi	65	19	29.2
Lita	33	12	36.4
Komanda	19	5	26.3
Bamboo	18	5	27.8
Kilo	9	1	11.1
Mangala	26	14	53.8
Tchomia	15	2	13.3
Damascus	5	0	0.0
Aungba	6	2	33.3
Rimba	3	0	0.0
Aru	3		33.3
Mambasa	2		50.0
Logo	7	1	0.0
Drodro	3	1	100.0
Gety		0	0.0
Kambala	1	3	100.0
Nia-Nia	2	0	41.7
Fataki		2	0.0
Mandima			25.0
Lolwa			100.0
Other areas not yet identified	12 1 4 1 17	5 0 1 1 0	0.0
NORTH KIVU	138	80	58.0
Butembo	40	18	45.0
Katwa	51	37	72.5
Blessed	29	17	58.6
Oicha	3	2	66.7
Kalunguta	2		50.0
Kyondo	4	1	50.0
Goma		2	0.0
Masereka	1	0	0.0
Vuhovi	2 1	0 1	100.0
Mabalako		0	0.0
Musienene	1	2	50.0
SOUTH KIVU	43	1	33.3
Miti-Murhesa	3	1	33.3
TOTAL	1,528	492	32.2

Provincial dynamics and case concentration

Over the past 24 hours, 26 new confirmed cases, including 9 community deaths, have been reported in the provinces of Ituri (21 cases, including 6 deaths) and North Kivu (5 cases, including 3 deaths). A total of 19 confirmed deaths have been recorded during this period, including the 9 community deaths and 10 deaths of previously confirmed patients. This development reflects the continued progression of the 17th Ebola Virus Disease (EVD) outbreak in the Democratic Republic of the Congo. To date, the cumulative total stands at 1,528 confirmed cases and 492 confirmed deaths, representing an overall case fatality rate of 32.2%.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **90.7%** of cases (n=1,387) and **83.5%** of deaths (n=411), representing a case fatality rate of **29.6%**, making it the absolute epicenter of the epidemic. The health zones of Bunia (431 cases), Rwampara (333 cases), and Mongbwalu (276 cases) remain the most affected, accounting for approximately 74.9% of reported cases at the provincial level and 68.2% at the national level. Within these same health zones, moderate case fatality rates were observed in Rwampara (20.7%) and Bunia (26.0%), while the Mongbwalu zone **also** has a high case fatality rate of **50.4%** (139 deaths out of 276 cases), suggesting persistent challenges in early intervention and access to care. It should be noted that 17 additional cases (with no associated deaths) have been reported while the health zones of origin remain to be identified, which could indicate a geographical spread requiring further investigation.

Severity of the epidemic in North Kivu

A total of 138 confirmed cases and 80 deaths have been reported in North Kivu province since the start of the outbreak, representing a **case fatality rate of 58.0%**. This province has the most concerning profile in terms of severity, with a case fatality rate significantly higher than the national average. This high case fatality rate is concentrated in the health zones of **Katwa** (72.5%), **Oicha** (66.7%), **Beni** (57.1%), **Kalunguta**, **Kyondo**, and **Musienene**. (50.0%) respectively. The Katwa health zone has the highest number of reported cases (n=51), followed by the Butembo health zone (n=40) with a case fatality rate of 45.0%. A case fatality rate of 100% was exceptionally observed in the Vuhovi health zone (which has a small population).

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri & Kinshasa

- Holding the coordination meeting with the response team and technical partners and financial support for field activities.
- Briefing of provincial supervisors on the general aspects of Ebola virus disease in the Bunia health zone.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of July 3, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Report (D-1)				60
New Alerts		0	4	1071
Total alerts for the day		768	11	1,131
Alerts investigated		768	15	902
Investigation rate		768	15 100.00%	79.7%
Alerts confirmed	Alive		5	118
	Deceased	56,292,348,119 34.20% 74,06,00% 39 22	0	67
Total suspected cases for today	119	61	5	185

- By the end of the day on July 3, 2026, **1071 alerts** had been reported across the three provinces, **902 (79.7%)** were investigated, **185 suspected cases including 67 deaths were identified** (Table 3).

Table 4. Contact tracing of confirmed cases as of July 3, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri*	7,467	6,148	82.3%
North Kivu	2,504	1978	78.9%
Total	9,971	8,126	81.5%

* The follow-up rate was calculated only for health zones that submitted reports (completeness: 90%). The health zones of Bunia, Aungba, and Nia-Nia (Ituri) did not have data available at the time of report consolidation.

- Continued in-depth investigations and exhaustive listing and systematic follow-up of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and followed per case).
- **329 new contacts** were listed, including **132** in 4 health zones in Ituri province and **197** in 3 health zones in North Kivu province.
- **44 contacts** have **become suspects** in the Ituri province.
- **501 contacts successfully completed their 21-day follow-up**, including **384** in 4 health zones in Ituri province and **117** in 2 health zones in North Kivu province.

Update on checkpoint and point of entry (PoC/PoE) activities

- **Ituri** : Five (6) alerts were notified to the PoCs, including 5 active at the Mudzibala PoC and 1 at the Ofoo PoE PoC. The alert was validated and referred to the Aru CTE.
- **North Kivu**: Interception of a lifeless body coming from Uganda to the DRC (Beni) at PoE Kasindi

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of July 2nd 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	98.1% (54/55)	100.0% (14/14)	98.5% (68/69)
Number of people who went through PoE/PoC testing,	95,658	52,921	148,579
% of travelers screened, % of	94.2%	97.5%	95.9%
travelers who washed their hands, % of	93.5%	97.5%	94.0%
travelers made aware of the issue	98.9%	97.5%	98.2%
Number of alerts notified	8	1	9
Number of validated live alerts	6	0	6
Number of lifeless bodies intercepted today	2	1	3
Number of problematic contacts intercepted today; %			0
of validated live alerts referred to CT/CTE; % of	0	0	66.6%
deceased bodies swabed	75%	0.0%	50.0%
Number of positive results of referred suspected cases	50.0% 0	0.0% 0	0

4.3. Laboratory

- **Ituri** : 54 samples collected and analyzed (**positivity 38.8%; n=21**).
- **North Kivu** : 93 samples collected and analyzed (**positivity 5.4%; n=5**).
- **South Kivu** : 5 samples collected and ongoing analysis.
- Installation of diagnostic laboratories in the health zones of Komanda, Nizi, Mambasa and Nia-Nia, in the province of Ituri.

4.4. Infection Prevention and Control (IPC)

Ituri: EDS: 77 alerts were notified, of which 54 were carried out, with 23 reported in the health zones of Bunia, Mongbwalu, Nyakunde, , Rwampara, Nizi, Komanda, and Bambu; 15/15 households were identified and decontaminated around confirmed cases; 10 means of transport (ambulances, hearses) were decontaminated, and 35 service providers were briefed on Ebola Virus Disease (EVD); 3 social and healthcare establishments (SHEs) were identified and decontaminated around confirmed cases in the health zones of Bunia, Rwampara, Mongbwalu, and Komanda; 4 SHE decontamination/biocleaning of public and specific places: BCZ office (Komanda), Akwa CSR (Damas health zone), Mongbwalu health zone, Nyakunde (10 mattresses); IPC/WASH assessment of 8 SHEs in the health zones of Mongbwalu, Bunia, Lita, Nyakunde, and Rwampara; Follow-up and support of 29 ESS: CM Bethel 42.9%; CM St Gabriel 31.7%, CM Lobiko 67.7%; CM Amani 33.9%; PS Baraka 27%, CM Ndirira 31%; CM Peniel 51%; Clin Espoir 56.5%; Decontamination of 10 means of transport (ambulances, hearses) in Nizi, Komanda, and Mongbwalu; and briefing of 80 community health workers and leaders in the Bunia (65) and Niania (15) health zones on general information about Ebola Virus Disease (EVD) and standard precautions; 15 decontamination operations around confirmed cases in the health zones of Bunia, Mongbwalu, Rwampara, Nyakunde, and Lita; 12 people were briefed in households during activities in the Mongbwalu health zone; IPC kits were distributed to the Kutumba Health Center and the Kalivuli Health Center, accompanied by a briefing from two hygiene specialists on the donning and doffing of personal protective equipment (PPE), hand hygiene, and decontamination of the patient's environment.

North Kivu: An IPC (infection prevention and control) assessment (scorecard) was conducted in 8 health facilities in the Beni health zone, with an average compliance score of 28.1%, reflecting a low level of implementation of essential infection prevention and control measures. Scores ranged from 19% at CM Mbanivuka to 53% at Dispensaire de l'Espoir, with the following results: Dispensaire Katoka (20%), CM La Joie (34.8%),

Imani Bora Dispensary (32.5%), CM Mbanasuvya (20%), CM Mbanivuka (19%), Dispensary of Hope (53 %), CM Kithimba (25.8%) and Dispensaire Tuha (20%); decontamination of 3 ESS after the passage of suspected Ebola cases and organized the briefing of 183 health providers on the main IPC themes during supervisions of priority structures

4.5 Holistic care

- Ten (10) deaths were recorded, including 8 in Ituri, among patients with Ebola virus disease in Ebola treatment centers in the provinces of Ituri (3 at the Ebola Treatment Center in Mongbwalu, 2 at the Ebola Treatment Center in ISTM, 1 at the Ebola Treatment Center in Bunia, 1 at the Ebola Treatment Center in Elikya and 1 at the Ebola Treatment Center in SOTA) and North Kivu (2 at the Ebola Treatment Center in Katwa).
- 10 patients with Ebola virus disease, including 9 in Ituri province (3 at CTE ISTM, 3 at CTE Mongbwalu, 2 at CT Bambu and 1 at CTE Nyakunde) and 1 at CTE Katwa in North Kivu province, have been declared cured after obtaining negative control tests.
- At the end of the day on July 3, 774 hot meals were served, including 150 to confirmed cases, 127 to suspected cases, 446 to PPLs and 52 to accompanying persons.

Table 1: Analysis of community deaths and hospital mortality during the Ebola Bundibugyo virus disease epidemic in the DRC as of July 3, 2026

Province	Zone de sante	Cas confirmés	Deces confirmés - statut final	Taux de létalité (Communautaire + intra Hosp)	Deces a la notification	Cas confirmés notifiés dans 24h	Deces confirmés parmi les notifiés dans 24h	Taux de létalité parmi les cas confirmés notifiés en 24h de DDS	Décès communautaire : Décès à la notification + Décès en 24h de notification	Taux de létalité (décès communautaire)	Cas confirmés notifiés après 24h	Deces confirmés parmi les notifiés après 24h	Taux de létalité parmi les cas confirmés notifiés après 24h de DDS
Ituri	Bunia	430	82	19.1%	80	108	20	18.5%	100	23.3%	282	53	18.8%
Ituri	Rwampara	326	59	18.1%	41	107	22	20.6%	63	19.3%	177	27	15.3%
Ituri	Mongbwalu	261	95	36.4%	95	60	24	40.0%	119	45.6%	189	66	34.9%
Ituri	Nyankunde	94	7	7.4%	6	34	2	5.9%	8	8.5%	57	5	8.8%
Ituri	Nizi	93	31	33.3%	30	19	9	47.4%	39	41.9%	47	11	23.4%
Ituri	Lita	41	16	39.0%	16	10	3	30.0%	19	46.3%	24	9	37.5%
Ituri	Mangala	30	12	40.0%	12	8	4	50.0%	16	53.3%	7	1	14.3%
Ituri	Komanda	22	6	27.3%	6	6	1	16.7%	7	31.8%	11	4	36.4%
Ituri	Bambu	19	3	15.8%	3	7	0	0.0%	3	15.8%	11	2	18.2%
Ituri	Tchomia	18	8	44.4%	4	4	1	25.0%	5	27.8%	13	7	53.8%
Ituri	Kilo	12	2	16.7%	2	3	0	0.0%	2	16.7%	7	2	28.6%
Ituri	Nia Nia	9	3	33.3%	3	2	0	0.0%	3	33.3%	5	2	40.0%
Ituri	Damas	7	2	28.6%	2	1	0	0.0%	2	28.6%	4	1	25.0%
Ituri	Aungba	5	2	40.0%	2	3	1	33.3%	3	60.0%	1	0	0.0%
Ituri	Mambasa	5	3	60.0%	3	1	0	0.0%	3	60.0%	1	0	0.0%
Ituri	Aru	4	1	25.0%	1	2	1	50.0%	2	50.0%	1	0	0.0%
Ituri	Mandima	4	1	25.0%	1	0	0	0.0%	1	25.0%	3	1	33.3%
Ituri	Drodro	3	3	100.0%	3	0	0	0.0%	3	100.0%	3	3	100.0%
Ituri	Rimba	3	0	0.0%	0	0	0	0.0%	0	0.0%	3	0	0.0%
Ituri	Logo	2	0	0.0%	0	1	0	0.0%	0	0.0%	1	0	0.0%
Ituri	Adi	1	1	100.0%	1	0	0	0.0%	1	100.0%	0	0	0.0%
Ituri	Fataki	1	0	0.0%	0	0	0	0.0%	0	0.0%	1	0	0.0%
Ituri	Gety	1	0	0.0%	0	1	0	0.0%	0	0.0%	0	0	0.0%
Ituri	Kambala	1	0	0.0%	0	0	0	0.0%	0	0.0%	0	0	0.0%
Ituri	Lolwa	1	1	100.0%	1	0	0	0.0%	1	100.0%	1	1	100.0%
Nord Kivu	Katwa	49	26	53.1%	26	40	23	57.5%	49	100.0%	6	2	33.3%
Nord Kivu	Beni	24	17	70.8%	12	5	5	100.0%	17	70.8%	15	9	60.0%
Nord Kivu	Butembo	19	9	47.4%	8	3	0	0.0%	8	42.1%	16	9	56.3%
Nord Kivu	Kyondo	2	1	50.0%	0	0	0	0.0%	0	0.0%	2	1	50.0%
Nord Kivu	Goma	1	0	0.0%	0	1	0	0.0%	0	0.0%	0	0	0.0%
Sud Kivu	Miti Murhesa	3	1	33.3%	1	0	0	0.0%	1	33.3%	1	1	100.0%
TOTAL	TOTAL	1,491	392	26.3%	359	426	116	27.2%	475	31.9%	889	217	24.4%

A total of 392 deaths were recorded, the majority (359) of which occurred within the community (community deaths). The overall community case fatality rate thus reached 31.9%. The geographical distribution shows a high concentration in Mongbwalu (119 deaths), Bunia (100 deaths), and Katwa (49 deaths). Patients either died within the community or presented at health facilities far too late.

Late consultation in healthcare facilities and isolation in the community

This significantly increases the risk of secondary transmission within households and among patients' contacts.

The in-hospital case fatality rate is slightly lower (24.4%), representing 217 deaths.

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of July 3, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	3 (100.0%)	5 (125.0%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	54 (49.5%)	12 (100.0%)	0
New suspected cases that benefited from SMSPS interventions: 13 (61.9%)		2 (100.0%)	0
Suspected cases under follow-up who received interventions: 71 (74.7%) SMSPS		17 (100.0%)	3 (100.0%)
Laboratory results have been announced.	9	28	0
Including positive ones	4	7	0
Number of children separated in daycare who benefited from interventions SMSPS	3	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	0	6
Number of accompanying persons at the CTE who benefited from interventions SMSPS	49	13	1
Number of PPLs who benefited from SMSPS interventions	36	12	1
Number of household and community members who benefited from SMSPS interventions	27	2	0
Number of EDS supported	9	7	0

4.6 Continuity of care

- 20 midwives and nurses from the Bambu health zone in Ituri were trained on sexual and reproductive health in the context of Ebola. • A total of **109 PPLs** were confirmed, including 34 deaths (31.2%) and **46 recoveries** in the province of Ituri.
- Launch of free healthcare in the 4 priority health zones (Bunia, Mongbwalu, Nyankunde and Rwampara); Strengthen the pillar (logistical resources, etc.); Strengthen infection prevention and control (IPC) measures in health establishments

4.7 Risk communication and community engagement

Ituri: 285 social mobilization activities (community dialogues and mass awareness) reaching 36,499 (11,228 men and 19,271 women) on preventive measures for Ebola Virus Disease, community-based surveillance, care, decontamination and EDS in the health zones of Mongbwalu, Nyankunde, Nizi, Rwampara, Nia-Nia, Aungba, Bambu, Aru; 400 educational talks and home visits organized by 282 Recos, 850 households were reached and 7,330 people reached (3,587 men and 3,743 women) in the health zones of Bunia and NiaNia; Briefing and training of 90 service providers on community-based surveillance, CREC with practical exercises on the organization of home visits, facilitation of educational talks in their respective environments and the use of tools (community definition of cases, contact registration form, contact follow-up form, and home visit form) in the health zones of Mongbwalu and Bunia; ; 18 broadcasts of spots and messages on preventive measures for Ebola Virus Disease (EVD) (community definition of cases, contact registration form, contact follow-up form, and home visit form); 18 broadcasts of spots and messages on preventive measures for EVD.

Community feedback: Related to patient care; in the various CTEs

Support for the pillars of the

response : Surveillance: support for the investigation and listing of contacts around 0 cases in Rwampara, 11CME, Shario.

PCI: Support for other pillars: surveillance, PCI, EDS in the NIANIA Health Zone

Preparation for the decontamination of 2 structures, 2 of which were carried out in 2 Tshunga and 1 Bandayi

Preparation for membership in the EDS of the 8 families 5 realis

North Kivu: 1,818 Community Health Workers (CHWs) visited 14,620 households and raised awareness among 36,601 people in the health zones of Goma, Karisimbi, Nyiragongo, Vuteste, Vulamba, Ngengere, and Kyangike, achieving 87% coverage against a target of 95%. (To be verified: the breakdown by sex does not correspond to the total announced.) Seven educational talks and awareness sessions were held with 915 people, increasing knowledge of warning signs from 43% to 81% and handwashing practices from 32% to 76%. 165 service providers (100 members of the Community Health Centers and 65 CHWs, community leaders, traditional healers, and managers) were trained, with practical skills improving from 52% to 88% and a validation rate of 96%.

4.8. Logistics

Ituri

- 26 vehicles and 4 ambulances (PNC, FARDC, HGR Bunia and CTE Rwampara) were put into service
arrangement of the different pillars to ensure their mobility towards the axes:
 - Development work continues at: CTE RWAMPARA
 - Delivery of PCI and PECH inputs to the SOTA CSR;
 - Delivery to HGR RWAMPARA CREC kit;
 - Delivery to SGI Prov PIC kit and surveillance; Nyankunde ZS 7 packages of 29 kg surveillance kit,
 - CTE CME kit PIC 11 packages of 70 kg; •
- Reception of PEC and PCI inputs from the WHO.

North Kivu

- Delivery to Beni: PCI kit 145 packages and PEC kit 1695 kg

4.9. Security

The security situation remains calm but unpredictable in Bunia and surrounding areas.

An act of vandalism occurred at the Irumu Centre police station by unidentified individuals following the arrest of one of their members by the security service.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Continuation of stakeholder briefing and community awareness activities in the Bunia and Rwampara, Mongbwalu, and Aru health zones. 114 stakeholders were briefed, including 58 women and 54 men, on PSEA; 360 FHI staff, including 16 participants (12 women and 4 men), were also briefed on PSA; 98 community health workers were sensitized at the IDP site; and awareness-raising was conducted for taxi drivers at the CME parking area in the Rwampara health zone.

A total of 18 participants (1 woman and 17 men), including staff from Istm-Nyankunde in the Rwampara Health Zone; a total of 40 participants (16 women and 24 men), taxi drivers and street vendors in the Bankoko neighborhood of Bunia; a total of 35 participants (17 women and 18 men), students from Istm-Nyankunde in the Rwampara Health Zone; a total of 169 participants (129 women and 40 men).

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity of CTEs and saturation in North Kivu (138.8%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3. Performance of the monitoring of contacts not optimal (81.3% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not yet broken down by health zone (17 case)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation, Remove any potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (96 cases) HCW (including 19 deaths, case fatality rate 19.7%)	Urgent supply, strengthening of IPC training
8 Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (54.2%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10. Insufficient coordination and low utilization of COUSP frameworks	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11. Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13 Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

6. PHOTOS OF FIELD ACTIVITIES



CAMON 30S •

23mm f/1.8 1/33s ISO2096

Sitrep development team in the final stages of validation



Briefing on Community-Based Surveillance in Bunia

**POUR TOUTE INFORMATION
SUPPLÉMENTAIRE, VEUILLEZ CONTACTER**

**Pour l'Institut National de la Santé Publique
(INSP) de la RDC**

**Le Directeur Général de l'INSP
Dr MWAMBA KAZADI Dieudonné**

E-mail : dieudonne.mwamba@insp.cd

Tél : +243 816 040 145

**Le Coordonnateur COU-SP
Prof NGANDU Christian**

E-mail : Christian.ngandu@insp.cd

Tél : +243998091915

**L'Incident Manager
Prof Pierre AKILIMALI**

E-mail : pierre.akilimali@insp.cd

Tél : +243815800288

**Avec l'appui technique de l'Organisation Mondiale
de la Santé (OMS), de Africa CDC et Bluesquare**